



JB³Ai / ta JB³

Jonathan Blackburn

jono@jonoblackburn.com

hi@jb3ai.com

+27 71 969 1848

Ash Medical Suites
177 Garsfontein Road,
Ashlea Gardens

Natural Medicinal Services (NMS)

Market Research, Brand, Website and Marketing Transformation Proposal

Prepared for: Natural Medicinal Services (Pty) Ltd / NMS leadership

Prepared by: Jonathan Blackburn

Date: 16 August 2026

Status: Phase-one strategic proposal based on eight supplied legacy documents and current external market research

Executive Summary

Natural Medicinal Services has a rare strategic asset: a repeatedly stated South African heritage dating to 1934, supported by a historically broad portfolio of herbal remedies, medicinal teas, raw botanicals, topical preparations, dietary products and imported ranges. The supplied records also show that the business once possessed meaningful trade infrastructure, including internal product codes, barcodes, NAPPI fields, wholesale vendor numbers, case-pack logic, suggested retail pricing, pharmacist involvement and relationships with pharmacy and wholesale channels.[1] [3] [5] [6]

The documents do **not**, however, constitute a reliable current operating record. They conflict on the legal entity and registration number, ownership spelling, contact details, domain, Swiss distribution dates, product names, reference codes, VAT calculations, prices, availability and ecommerce policies.[1] [3] [5] [6] [7] [8] Several product descriptions make direct or implied disease-treatment, cure, detoxification, blood-pressure, diabetes, infection, pregnancy, slimming and other high-risk claims, while the same material acknowledges that some herbs may be toxic or carcinogenic and that no herb can be said to cure an ailment.[1] The new-site product document has 2026 file metadata but materially reproduces the legacy 2017 catalogue and its unresolved errors.[2] [8]

The market opportunity is attractive but execution-intensive. South African industry research reports 9.0% current-price growth in dietary supplements from 2023 to 2024 and forecasts an 8.1% compound annual growth rate from 2025 to 2029, although current-price growth includes inflation.[12] Gauteng remains the strongest launch geography, with an estimated 15.83 million residents in 2024, representing 25.1% of the national population. [10] Pharmacy groups and natural-health retailers provide substantial reach, but national chains are highly promotional, data-led and increasingly influenced by private-label economics.[18] [19] Online retail is also growing rapidly, yet mobile usability, delivery reliability, payment friction, trust, claims compliance and customer acquisition remain decisive.[22]

The recommended position is **“Johannesburg herbal heritage, made accountable.”** This means translating traditional botanical knowledge into affordable modern self-care through verified provenance, named ingredients, controlled claims, batch-level quality, clear dosage and safety information, pharmacist or qualified-professional escalation, and respectful coexistence with conventional medicine. The phrase is a strategic direction, not final consumer copy. The 1934 claim should be used publicly only after documentary substantiation; if continuity cannot be proved, the wording should be qualified, for example as “roots traced to 1934,” subject to legal approval.

The transformation should not begin with a cosmetic logo exercise or wholesale migration of the historic catalogue. It should proceed through six gated phases over approximately 18 months. The first ten weeks should establish corporate, product, regulatory and commercial truth while completing customer, channel, competitor and demand research. The next four months should define the brand architecture, select three to five compliant hero products, create the identity and packaging system, and build a controlled product information and digital platform. A 90–180 day Gauteng pilot should then test the hero portfolio through NMS direct-to-consumer channels, one specialist ecommerce partner and 15–30 independent pharmacies before wider rollout.

A realistic planning allowance for the core 18-month transformation is **R2.9 million to R5.6 million excluding VAT**, excluding major manufacturing upgrades, product reformulation, SAHPRA application fees, large packaging print runs, inventory, retailer listing fees, consumer media and national rollout. A lean validation-first route can start with **R650,000 to R1.15 million** for the first ten weeks plus essential compliance triage. Management should approve the full transformation only through stage gates, releasing later investment after corporate truth, compliant hero-product selection and pilot economics are demonstrated.

Primary recommendation: protect the heritage, but rebuild the business around verified facts, controlled product data, responsible claims and commercial proof. NMS should become a trusted botanical authority rather than a catalogue of unverified historical remedies.

1. Scope, Method and Evidence Standard

This proposal answers four questions: what the historical documents prove about NMS; what must be validated before the business can be reintroduced; where the current South African market opportunity

lies; and how NMS should sequence market research, identity, website and marketing work into manageable mini-projects.

The internal evidence base comprised eight supplied PDFs covering 2016–2017 corporate profiles, brochures, product-copy drafts, price lists, wholesaler data and online-shop terms. [1]–[8] The external phase reviewed current South African population, market, regulatory, ecommerce, pharmacy, wholesaler, retailer, competitor and brand-benchmark sources, with priority given to Stats SA, SAHPRA, South African legislation, government departments, current company reports and live retailer evidence.[9]–[32] Expert and firsthand video material was used as contextual evidence and cross-checked against official sources; no video claim is treated as law or proof of product efficacy.[30]–[32]

Three evidence labels govern this report. **Verified historical fact** means the statement appears in the supplied business documents and is reported with its date context, but may no longer be current.

Externally verified fact means it is supported by a cited current public source. **Recommendation or hypothesis** means it is a proposed strategic direction that must be tested or approved.

This proposal is not a legal, clinical, tax or regulatory opinion. Product classification, licensing, labels, claims, registrations, advertising, ecommerce terms, privacy, VAT and cross-border sales require qualified South African advisers and product-specific decisions.

2. What the Documents Reveal About the Business

2.1 Verified historical business profile

The documents consistently present Natural Medicinal Services as a Johannesburg-based South African business dealing in herbal and natural preparations, with the phrase “Established 1934” used as the principal heritage signal.[1] [3] [5] Historical profiles describe local manufacture, selection and sourcing of herbal ingredients, pharmacist or industrial-pharmacy leadership, more than 150 simplex herbs, and distribution of Swiss ranges including Hermes, Dr Ernst Richter’s and Kernosan or Kennosan.[1] [3] [5] The portfolio extended beyond remedies into tea blends, raw herbs, supplements, beverages, salt and sweetener substitutes, ointments, hair products, mouthwash and tissue salts.[3] [5]

The records show a historically mixed business model. Registered vendors received price lists and suggested retail prices; trade enquiries were actively invited; products were advertised as available through pharmacies and health shops; and wholesaler records listed Alpha Pharm, Dis-Chem, Transpharm, Shoprite/Transfarm, UPD and Pharmed.[1] [3]

[6] Separately, draft online-shop terms describe a direct-to-consumer site using South African-rand prices, card or PayPal processing, bank transfer and worldwide courier delivery.[7] This proves that both trade distribution and ecommerce were contemplated, but it does not establish which model is currently operational.

2.2 Historical portfolio architecture

The legacy range can be organised into seven portfolio groups. These groups are a starting taxonomy only; every product must be validated as active, legal, available and commercially viable before inclusion.

Portfolio group	Historical content	Required decision
Finished oral remedies	Lexan, Fo-Ti-Tieng, Cystosan, Antifect, Skin Clear, Vegetex, Boldo, blood-pressure, arthritis, strength and other capsules,	Verify formula, code, active status, classification, claims, warnings, margin and brand condition-led tablets, ownership powders or liquids
		Separate owned from
	Nature's Own 70 g blends, Medicinal teas and compounds, Hermes numbered teas and Ernst Richter's tea indications	distributed ranges; normalise Dr variants and permitted
Raw herbs and simplex botanicals	claimed range exceeding 150 herbs with common and botanical names	Build a separate raw-herb data A model including plant part, grade, form, origin, testing and safe-use restrictions
Topical products	Aloe Vera, Bulbinella, Chickweed, Comfrey, Garlic and Marigold ointments plus creams and claims with particular care	Review wound, infection, fungal, burn and therapeutic other legacy ointments
Hair, scalp and oral care	Birch and Nettle products, shampoo, tonic, hair grower mouthwash current formulations	Confirm cosmetic versus medicine classification and and
Herb-A-Lick, Herbalic, Alfalfa, substitutes	Dandelion beverage, allergens, suitability, claims Torula Yeast and ginseng	Verify dietary attributes, Nutrition, beverages and food/medicine products classification
Tissue salts and imported ranges	tissue salts, Kernosan/Kennosan, Hermes and Dr Richter products	Confirm rights, territory, Schuessler importer/distributor responsibility and current authorisation

2.3 Historically demonstrated commercial capability

The price and channel records demonstrate that NMS previously maintained a reasonably structured trade catalogue. Product lines carried internal codes, barcodes, mostly populated NAPPI fields, ex-VAT and VAT-inclusive prices, and unit or case configurations including x6, x12 and x144.[4] [6] A Pharmed-specific column and discount calculations indicate account pricing, while vendor records show channel administration across Gauteng, Eastern Cape, KwaZulu-Natal and Western Cape.[6]

This is valuable institutional evidence, but not a current product master. The 2017 documents contain inconsistent VAT-inclusive values, incorrect discount calculations, spelling conflicts, pending NAPPI fields and a materially implausible Lexan case-price line. [4] [6] No price, discount, customer record or barcode should be loaded into a new website, ERP or retailer submission without reconciliation.

2.4 The central conclusion from the document audit

The historical business appears to have combined **manufacturing, importing, distributing, trade supply, product education and limited consumer commerce**. That breadth is a strategic advantage, but the old documents mix corporate narrative, medical claims, prices, pack sizes, codes, stock notes and policies in uncontrolled static files. The transformation therefore requires a controlled source of truth before it requires a new front end.

3. Critical Issues That Must Be Resolved

3.1 Corporate identity and continuity

The supplied records conflict on the current legal identity. One 2017 profile displays registration number **2007/017961/07**, while a price-list and licence-correspondence document uses **Natural Medicinal Services TVL. (Pty) Ltd** and registration number **1980/008685/07**; the online terms contain an incomplete registration number beginning with 2011.[5] [6] Ownership is described as Mvelo or Myelo Pharmaceuticals Group, and the acquisition is merely called “recent” in 2016–2017 material.[1] [3] [5]

The 1934 heritage may be commercially powerful, but the relationship between the original enterprise, the 1980 entity, the 2007 entity and the present operation has not been established. The company must produce a legal succession and ownership timeline before the date is placed at the centre of the rebrand.

3.2 Product and claims risk

The old catalogue includes numerous statements relating to disease treatment, detoxification, blood pressure, diabetes, infection, reproductive health, pregnancy, cardiovascular disease, liver disorders, weight loss, wounds and other material health outcomes.[1] [5] [8] The same documents contain selected interaction and pregnancy warnings, but most products lack complete directions,

contraindications, age limits, interaction guidance, allergens, adverse effects or current regulatory status.[2] [8]

SAHPRA identifies Western Herbal Medicine and health supplements within Category D, and product treatment depends on composition, dose, dosage form, intended use, claims, scheduling, registration or call-up status, licence scope, label content and evidence.[9] [33] Current public guidance requires product-specific analysis; neither an old F/R number nor a NAPPI field proves present authorisation.[9] Advertising may not exceed the accepted evidence or approved information, and public digital content is part of the practical advertising risk perimeter.[33]

3.3 Brand fragmentation

The business name, product families and imported brands are not coherently related. NMS, Nature's Own, NU Organic Remedies, Lexan, Hermes, Dr Richter, Kernosan/Kennosan and Brook Parker appear without a controlled hierarchy.[1] [3] [5] The visual materials vary between green/teal and green/orange palettes, several logo renderings, inconsistent typography and low-resolution scans. Historical slogans such as "Safe — Simple — Effective" and "herbal therefore harmless" conflict with the documents' own warnings and should not be reused.[1]

3.4 Digital and policy weakness

The online-shop terms are unfinished and stale. They leave the courier name, address correction period and registration details incomplete; use inconsistent payment statements; provide ambiguous contract and returns timing; and do not meet a modern privacy, cookie, data-retention, marketing-consent or processor-disclosure standard.[7] ECTA requires prescribed supplier, product, price, payment, delivery, security, privacy and returns information in covered electronic transactions, while POPIA applies to customer, prospect, online-identifier and special personal information.[27] [28]

4. External Market Assessment

4.1 Market attractiveness

The South African category is sizeable, fragmented and difficult to measure because packaged supplements, complementary medicines, traditional practitioners and informal medicinal-plant trade overlap. Commercial estimates vary and should not be added together. The most useful local directional evidence reports 9.0% current-price growth for dietary supplements from 2023 to 2024 and forecasts an 8.1% CAGR from 2025 to 2029.[12] Euromonitor's public summaries describe preventative wellbeing, plant-based preference, pharmacy importance, affordability pressure and fragmented competition, while noting that packaged-product measures exclude much informal trade.[13] [14]

Consumer behaviour is shaped by access, affordability, family tradition, word of mouth and perceived side-effect differences. A 2023 qualitative South African study also found concurrent herbal and

prescription-medicine use and low disclosure to clinicians; its small, heavily female sample explains behaviour but does not provide a national prevalence estimate.[15] This finding supports an NMS trust proposition built around interaction cautions and pharmacist escalation rather than a simplistic “natural means safe” message.

4.2 Geographic focus

Stats SA estimated South Africa’s 2024 population at 63.02 million, with Gauteng accounting for 15.83 million people or 25.1%.[10] Census 2022 counted 17.8 million households, including 5.3 million in Gauteng, while Gauteng, KwaZulu-Natal and Western Cape together contained approximately 56% of residents.[11] These data support a Gauteng-first pilot followed by KwaZulu-Natal and Western Cape, rather than immediate national rollout.

4.3 Channel reality

Clicks reported 990 stores and 780 pharmacies for FY2025. Promotional sales represented 47.0% of turnover and private label accounted for 30.6% of front-shop sales, demonstrating the scale, promotional pressure and margin competition that a small external brand faces. [18] UPD reported 96.2% availability and 97.3% on-time delivery, providing a useful distribution-service benchmark.[19] Dis-Chem’s FY2026 report described 316 pharmacies and a 280-pharmacy The Local Choice network.[20]

These chains matter, but they should not be the first proof point. Independent pharmacy groups, health shops, specialist ecommerce, NMS direct commerce and a tightly controlled marketplace test offer a more realistic initial learning environment. A wholesaler can increase efficiency; it cannot substitute for consumer pull, compliant product education, retailer support and rate of sale.

4.4 Digital opportunity

South African online retail reached an estimated R96 billion in 2024, up 35%, with further growth projected for 2025; health and beauty was among the growing online segments.[22] Live retailer review shows that natural-health shoppers are trained to expect visible prices, stock status, product filters, reviews, delivery thresholds, exact ingredients, preparation guidance, warnings, certifications and easy mobile purchasing.[23] [24] [25]

The initial digital opportunity is not a generic “herbal medicine” website. It is a controlled information and commerce system connecting ingredient, compliant need state, format and location. Examples include ingredient pages for dandelion or mullein; carefully governed collections for sleep and calm, digestion or seasonal wellness; format pages for loose herbs, teas, capsules and topicals; and location intent around Johannesburg or

Gauteng. Search volumes have not yet been validated and must be measured with South African Keyword Planner, Search Console and paid research tools before final prioritisation.

4.5 Competitive white space

The reviewed landscape includes direct brands such as Natura, Phyto-Force, A.Vogel, Feelgood Health, BuchuLife and Pegasus; supplement and botanical substitutes such as Solgar, African Extracts and Escentia; and channel competitors including Dis-Chem, Wellness Warehouse, Faithful to Nature, Living Naturally and Takealot.[23] [24] [25] [34]–[40]

No reviewed competitor visibly combines four capabilities in one coherent proposition: **verifiable long South African heritage, a raw-herb-to-finished-remedy portfolio, transparent batch-quality and safety information, and a modern trade portal**. This is the most credible whitespace for NMS, but only if those capabilities are real and documented.

5. Strategic Direction

5.1 Recommended positioning

Strategic positioning: Johannesburg herbal heritage, made accountable.

NMS should present itself as a South African botanical authority that translates traditional knowledge into affordable modern self-care through verified provenance, named ingredients, controlled manufacturing and quality information, clear dosage and safety guidance, and respectful collaboration with conventional healthcare. The position should lead with accountability rather than with unsupported efficacy.

This direction deliberately avoids four weak positions. NMS should not compete as a generic “natural wellness” brand, because that market is crowded. It should not compete mainly on cheap prices, because chains and private labels possess stronger scale. It should not present heritage as proof of efficacy, because longevity and clinical evidence are different. It should not use anti-pharmaceutical language, because consumers who combine herbal and prescribed products need cooperation and safe referral, not opposition.

5.2 Brand architecture hypothesis

The working hypothesis is an **endorsed masterbrand**:

Layer	Proposed role	Decision required
NMS / Natural Medicinal Services	Corporate trust mark, manufacturer/distributor identity and quality authority	Confirm exact legal and public name
Nature's Own or another chosen consumer architecture	Consumer-facing range or family system	Test awareness, ownership, trademark status and retailer response
Need-state families	Five to eight clear, compliant families such as Daily Balance, Digestive Comfort, Seasonal Support, Calm & Rest, Mobility and Topical Care	Final names require claims review and research
Product descriptor	Plain ingredient, format and pack description	Must support accurate search and reduce medication-error risk
Distributed brands	Clearly marked third-party brands with role, rights and territory explained	Confirm agreements and current authorisation

The final brand name and family structure should be selected after management interviews, trademark/domain checks, consumer testing and the product-claims matrix. The proposal does not recommend discarding Nature's Own; it recommends testing whether it still owns useful recognition.

5.3 Value proposition and proof hierarchy

The final proposition should be built from proof in descending order of reliability:

Proof level	Appropriate evidence	Communication rule
Corporate history	Archival advertisements, registrations, invoices, packaging, premises and succession records	State only what the evidence proves

Product identity and quality	Ingredient identity, plant part, supplier qualification, contaminant testing, batch release, stability and traceability	Show process and date, not generic badges
Traditional use	Named tradition, source and historical context	Do not present as clinical proof
Ingredient evidence	Pharmacopoeial or literature evidence relevant to ingredient, dose and format	Do not automatically transfer to the finished product
Product-specific evidence	Accepted observational, user or clinical evidence	Distinguish study type and sample size
Approved/permitted claims	Exact product-specific wording approved by the regulatory governance process	All channels must publish only controlled wording

6. Priority Segments

The first launch should not target “everyone interested in natural health.” It should focus on a small number of segments whose needs match the proposed trust system.

Priority	Segment	Need	NMS response	Validation method
1	Value-conscious Gauteng household health buyers	Affordable, understandable support for everyday low-risk needs	Trial sizes, transparent pack economics, clear directions and pharmacy or WhatsApp help	Survey, interviews and pilot sell-through
2	Ingredient-aware online shoppers	Exact botanical, provenance, format, stock, price and delivery information	Searchable ingredient library and strong product pages	Keyword research, usability tests and DTC behaviour

3	Adults using both herbs and prescribed medicines	Interaction guidance and confidence about when to consult a professional	Safety content, medication cautions and pharmacist escalation	Consumer and pharmacist interviews
4	Independent pharmacists and health-shop buyers	Compliant products, reliable supply, sell-through support and evidence files	Retailer data room, training, merchandising and replenishment	Buyer interviews and 15–30-store pilot
5	Raw-herb and practitioner buyers	Botanical identity, plant part, grade, origin, specification, pack sizes and COAs	Verified trade portal and technical catalogue	Existing-customer audit and account pilot

7. Portfolio Strategy

7.1 Start with three to five hero products

The first launch should use **three to five hero SKUs**, expandable to six to ten only after commercial and compliance proof. A broad catalogue would increase packaging cost, content risk, inventory complexity and retailer resistance before NMS understands present demand.

Each candidate product should be scored against the following weighted criteria:

Criterion	Weight	Pass condition
Regulatory and claims readiness	25%	Clear classification, defensible indication, complete label/PI/PIL path and licence coverage
Existing demand and repeat potential	15%	Verified sales, reorders, retailer interest or strong research evidence

Gross contribution after channel costs	15%	Attractive after VAT, trade margin, promotion, fulfilment, returns and marketplace fees
Supply and quality resilience	15%	Stable ingredient identity, dual sourcing, testing, lead time and shelf life
Brand differentiation	10%	Heritage, format, provenance or safety advantage that competitors do not easily copy
Retail operability	10%	Barcode, case pack, shelf legibility, forecastability and low error risk
Digital discoverability	5%	Clear ingredient or format intent and content potential
Cross-sell and regimen fit	5%	Complementary use without unsafe or unapproved recommendations

High-risk historical product areas such as serious cardiovascular, diabetes, fertility, infection, pregnancy/labour and cure-oriented claims should not be used merely because their old names are memorable. Lower-risk everyday categories may offer a faster lawful route, but selection must be based on the SKU audit rather than assumption.

7.2 Product information standard

Every active product should receive a controlled record containing canonical product and brand name; legacy aliases; active or discontinued status; formulation and quantities; common and botanical names; plant part; dosage form; pack and case variants; barcode; internal code; NAPPI status; classification; approved claim; directions; warnings; interactions; pregnancy, age and allergen guidance; storage; batch and shelf-life details; images; stock; price book and effective date; channel eligibility; and content owner and approval date.

8. Transformation Roadmap



The roadmap contains six phases and four investment gates. Phases overlap where work can proceed safely, but no downstream publication or scale decision should bypass the gates.

Phase 0 — Mobilise and Preserve, week 0–1

The first week should establish a sponsor, transformation lead and secure data room. All physical and digital records should be preserved, including old packaging, labels, invoices, catalogues, registrations, licence correspondence, agreements, photographs, sales data, customer records and domain credentials. Nothing should be deleted merely because it is outdated; the heritage archive may become valuable once authenticated.

Outputs: signed project charter, governance calendar, data-room index, stakeholder map, risk escalation route and document-preservation protocol.

Phase 1 — Establish Corporate, Product and Compliance Truth, weeks 1–6

This phase resolves the legal entity, ownership, directors, licences, facilities, manufacturing and distribution roles, address, controlled domains and 1934 succession story. In parallel, the team creates a provisional SKU master, maps active versus inactive products, and performs a red-amber-green claims and safety triage.

Gate A: management may proceed to public brand development only when the contracting entity, approved company facts, licence map and active-product universe are signed off.

Phase 2 — Validate the Market and Commercial Model, weeks 2–10

The market phase combines quantitative research, interviews, customer and account analysis, pricing, channel economics, keyword research and competitor testing. The work should not merely ask whether people like “natural products”; it should measure use, spend, purchase channel, trust signals, price sensitivity, desired format, medication co-use, objections, awareness of NMS and willingness to try specific compliant propositions.

Gate B: three to five hero SKUs are selected only after regulatory readiness, demand, supply, margin and channel fit are scored.

Phase 3 — Define Brand, Architecture and Experience, weeks 7–14

The brand strategy converts verified facts and market evidence into purpose, positioning, audiences, architecture, messages, tone, naming and proof. Customer and trade journeys are mapped, and the website scope is decided as catalogue, stockist, trade portal and/or ecommerce based on operational readiness.

Phase 4 — Build Identity, Packaging and Platform, weeks 12–26

The identity system, priority packaging, product photography, copy, policy suite, PIM/CMS, website, CRM, analytics and trade materials are produced. The site may launch with a catalogue and stockist/trade journey before full ecommerce if inventory, payments, shipping, service or compliance is not ready.

Gate C: launch requires approved identity and packaging, complete product records, policies, analytics, support and adverse-event routes, operational testing and executive sign-off.

Phase 5 — Pilot Gauteng, months 7–¹

Run a 90–180 day pilot with the same hero SKUs across NMS mobile DTC/WhatsApp, one specialist ecommerce partner, one marketplace only where contribution works, and 15–30 independent pharmacies. Use common SKU identifiers and weekly performance data.

Gate D: wider scale occurs only if sell-through, contribution margin, repeat purchase, fill rate, stockouts, returns, reviews, acquisition cost and safety indicators meet agreed thresholds.

Phase 6 — Scale What Works, months 10–18

Expand proven SKUs to KwaZulu-Natal and Western Cape, add carefully selected products, and approach larger chains with pilot evidence. National chain submissions should be bounded store-cluster proposals rather than requests to list the entire legacy range.

ID	Mini-project	Key deliverables	Timing	Primary owner
MP01	Corporate truth and succession	CIPC pack, entity and ownership map, directors, addresses, approved fact sheet, heritage evidence standard	Weeks 1–4	Executive + legal
MP02	Heritage archive	Digitised artefact register, dated timeline, provenance notes, rights and gaps	Weeks 1–8	Brand lead + archivist

¹ . Mini-Project Portfolio and Deliverables

MP03	Licence and operating-model audit	Facility/contractor map, licence scope, responsible person map, quality obligations	Weeks 1–6	Regulatory lead
MP04	Master SKU and brand register	Canonical names, aliases, ownership, codes, barcodes, NAPPI, variants, active status	Weeks 1–8	Product owner
MP05	Claims, label and safety audit	SKU classification, approved claims, evidence, directions, warnings, PI/PIL and quarantine list	Weeks 2–12	Pharmacist + regulatory counsel
MP06	Commercial and channel audit	Sales/margin by SKU/customer, price books, trade terms, MOQs, listings, fill rate, returns	Weeks 1–8	Finance + sales
MP07	Consumer research	600–1,000 respondent survey and qualitative interviews across priority provinces	Weeks 3–9	Research partner
MP08	Buyer and practitioner research	20–30 interviews with pharmacists, buyers, health shops, practitioners and clinicians	Weeks 3–8	Commercial lead

MP09	Competitor, digital and keyword research	Price and proposition map, SEO demand,	Weeks 2–8	Strategy + digital
		retailer search, channel requirements, benchmark UX		
MP10	Portfolio prioritisation	Weighted heroSKU scorecard, launch architecture, hold/retire/refor mulate decisions	Weeks 8–10	Steering committee
MP11	Brand strategy and naming	Positioning, architecture, messaging, tone, naming, trademark and domain decisions	Weeks 8–14	Brand lead
MP12	Visual identity and design system	Logo suite, heritage device, palette, typography, photography, illustration, accessibility and templates	Weeks 12–18	Design partner
MP13	Packaging system	Master structure, regulatory information hierarchy, family coding and 3–5 hero artworks	Weeks 14–22	Design + regulatory
MP14	Digital blueprint and PIM	Requirements, data model, journeys, architecture, integrations, governance and backlog	Weeks 10–16	Product/technology lead

MP15	Website and trade portal	Responsive site, catalogue, quality centre, stockist, trade application, optional	Weeks 16–26	Digital partner
		checkout, analytics		
MP16	Policy, safety and service system	POPIA, PAIA, cookies, sale/use terms, returns, complaints, adverse events, recalls and SLAs	Weeks 12–24	Legal + operations
MP17	Content and asset production	Product copy, expert articles, heritage content, photography, technical sheets and retailer assets	Weeks 16–26	Content lead
MP18	Gauteng pilot	Store recruitment, ecommerce activation, staff training, campaign, sampling and dashboard	Months 7–9	Growth + sales
MP19	Scale and optimisation	Regional expansion, chain pilot, portfolio extension, retention and quarterly reviews	Months 10–18	Executive + growth

10. Market Research Plan

10.1 Research questions

The first research phase should identify who buys herbal products, for which low-risk needs, in what forms, from which channels, at what price, and with what trust concerns. It must determine how the NMS name, Nature's Own, the 1934 story, Johannesburg origin, pharmacist involvement, local manufacture, botanical transparency and imported ranges influence choice. It must also establish what current customers and trade accounts value, which products still move, why accounts lapsed, and what prevents relisting.

10.2 Recommended research design

Module	Sample or method	Decision supported
Internal commercial analysis	24–36 months of sales, margin, returns, stock, expiry and customer data where available	Hero portfolio, account priority and economic baseline
Consumer survey	600–1,000 respondents across Gauteng, KZN and Western Cape, stratified by age, sex, income and urbanicity	Segment sizing, category behaviour, price and proposition testing
Consumer depth interviews	12–18 current, lapsed and category buyers	Language, trust, barriers, safety and journey insight
Buyer interviews	10–15 pharmacists, health shops, ecommerce and retail buyers	Listing criteria, margin, training, service and packaging expectations
Practitioner/clinician interviews	6–10 qualified stakeholders	Referral, interaction and education needs
Competitive shelf/digital audit	10–15 competitors across pharmacy, specialist and marketplace channels	Position, price, proof and design whitespace
SEO and demand study	South African Keyword Planner, Trends, Search Console and SERP analysis	Site taxonomy, hero content and media priorities
Concept test	Two or three positioning territories and packaging directions	Brand selection before expensive production
Pilot analytics	Weekly DTC and store-level performance for 90–180 days	Scale, revise or stop decisions

10.3 Required output

The research phase should produce a market model with explicit scope and overlaps; segment profiles; jobs-to-be-done; purchase journeys; a competitor and price architecture; customer and buyer verbatims; brand-concept test results; keyword clusters; a channel opportunity matrix; and a decision-ready hero-product scorecard. Findings should be tied to decisions, not presented as an isolated research document.

11. Brand Identity Programme

The identity should preserve authentic heritage while avoiding faux apothecary nostalgia. The historic circular NMS emblem, green botanical cues and “Established 1934” device may be retained or redrawn only after ownership and heritage substantiation. The design system should feel credible in a pharmacy, legible on mobile and accessible to older consumers.

The visual brief should require one strong master mark, restrained colour families, highly legible product names, clear dosage-form and pack-size differentiation, disciplined botanical illustration or photography, and a consistent evidence and warning hierarchy. Packaging similarity should be tested to reduce selection errors across products with related colours or names.

The voice should be calm, plain, precise and respectful. It should say what a product is, how it is used, what support it may lawfully offer, who should not use it, and when professional advice is appropriate. It should avoid superlatives, cure language, vague “detox” promises, anti-medical rhetoric and the implication that “natural” means harmless.

12. Website and Digital Platform Proposal

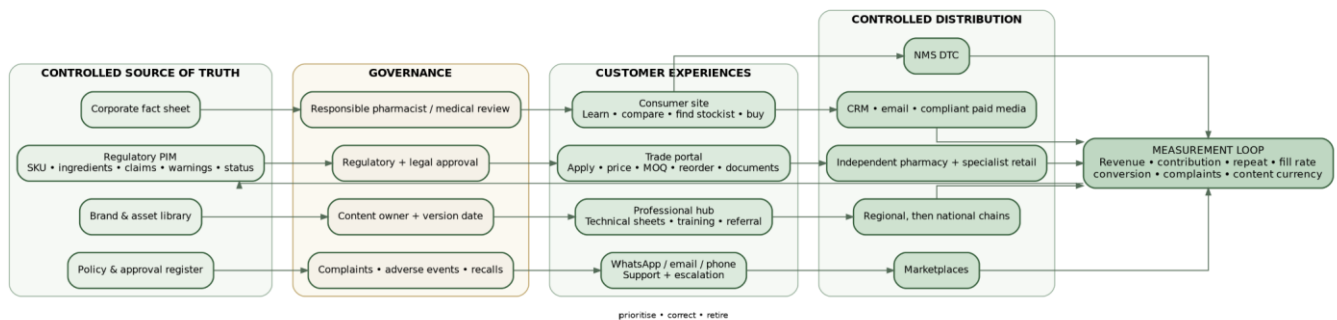
12.1 Recommended role of the website

The new website should be the controlled public expression of the business, not a digitised brochure. It should serve four connected functions: corporate trust, safe product discovery, trade enablement and measurable customer service. Ecommerce should be activated only when stock, pricing, payment, delivery, returns, support and product-level compliance are ready.

12.2 Information architecture

Section	Purpose	Essential content
Home	Establish relevance and trust	Verified proposition, hero range, quality proof, heritage preview, stockist and trade routes
Shop or Products	Safe product discovery	Browse by approved need state, ingredient, format, range and availability
Product detail	Informed choice	Ingredients, strength, form, pack, directions, warnings, interactions, regulatory status, stock, price, delivery and support
Herbs A–Z	Raw botanical authority	Common and Latin name, plant part, source, form, traditional context, preparation and safety
Our Story	Verified heritage	Dated timeline, people, places, artefacts, ownership transitions and current mission
Quality & Safety	Make accountability visible	Sourcing, testing, manufacturing, batch release, complaints, adverse events and recalls
Learn	Build evidence-led demand	Expert-reviewed herb guides, format comparisons, interaction and red-flag education
Where to Buy	Convert indirect demand	Verified stockists, locations, contact and product availability where possible
Trade & Professionals	Support B2B	Account application, technical files, MOQ, pricing, stock, training and reorder
Help & Policies	Reduce risk and service load	Shipping, returns, payment, privacy, cookies, terms, accessibility and complaints

12.3 Operating model



The website, retailer feeds, marketplace listings, social content and trade documents must publish from a controlled product-information record. Regulatory, medical and content approvals should be retained with version dates. Customer service, reviews, social comments and marketplace questions should feed complaints, adverse-event and productquality workflows rather than remaining isolated marketing data.

12.4 Functional requirements

The initial platform should be mobile-first and low-data. It should support powerful search; product filters; stock and availability; transparent ZAR pricing; guest checkout if ecommerce is activated; card and EFT options; WhatsApp support; account applications; a stockist finder; back-in-stock notices; delivery estimates; controlled reviews; document downloads; CRM consent; and analytics. Product, Offer, Breadcrumb, Organization/LocalBusiness and applicable shipping and return structured data should be implemented using Google's current documentation.[29]

The trade portal should support verified access, current price books, MOQ and case data, stock, reorder, technical sheets, pack images, training, merchandising files and account assistance. Public and trade data should come from the same master records while respecting channel-specific prices and permissions.

12.5 What not to build in version one

Version one should not attempt an ungoverned catalogue of every legacy product, a symptom checker, automated medical recommendations, international checkout for all countries, unrestricted user-generated health claims, or a complex loyalty scheme. These features add risk before NMS has established a clean product and service foundation.

13. Marketing Plan

13.1 Marketing objective

The first-year objective is to rebuild trust and prove repeatable demand for a small, compliant portfolio. Awareness is useful only if it converts into qualified product discovery, trial, repeat purchase, retailer reorder and improved account access.

13.2 Channel strategy

Channel	Role	First-year activity	Control requirement
Owned website and SEO	Long-term authority and conversion	Product pages, Herbs A–Z, heritage archive, safety content and local intent	Controlled claims and named reviewers
CRM and email	Repeat and education	Welcome, replenishment, education, stock and service messages	POPIA basis, consent records and suppression
WhatsApp and customer care	Assisted conversion and support	Product-navigation help, order support and escalation	Approved scripts, record handling and adverse-event training
Independent pharmacy and health shops	Trust and physical trial	Staff training, shelf tools, sampling and reorder support	Approved materials and weekly sell-through
Specialist ecommerce	Reach ingredient-aware shoppers	Rich listings, reviews and targeted promotions	Product-feed governance and contribution analysis
Marketplace	Demand test	Limited hero-SKU listings and search ads	Margin, stock and review monitoring
Paid search and social	Capture and test demand	Brand, ingredient, format and compliant need-state campaigns	Claim library, landing-page consistency and conversion tracking
PR and heritage content	Earn credibility	Archive launch, Johannesburg story, quality transformation and experts	Verified facts; no efficacy-by-heritage implication
Practitioners and pharmacists	Education and referral	Technical sessions, safety sheets and professional portal	Credential checks and no prohibited promotion

13.3 Content pillars

The recommended content mix is **Responsible Herbal Education, Johannesburg Heritage, Ingredient Provenance, Quality and Safety, Everyday Self-Care** and **Where to Buy**. Product promotion should be a minority of the content system until trust and discovery content are established.

Expert content should explain preparation, tea versus capsule or tincture, whole versus cut or powdered herbs, safe duration, contraindications, pregnancy and child cautions, medicine interactions and when to see a clinician. International or competitor copy must not be imported without South African review.

13.4 Pilot campaign

The Gauteng pilot should run in three waves. The first four weeks establish availability and train staff. The next six to eight weeks generate trial through compliant search, local social, sampling, retail education and creator content focused on product experience rather than medical outcomes. The final phase focuses on replenishment, review acquisition, buyer proof and unit economics.

14. Measurement Framework

The transformation needs one executive dashboard connecting corporate readiness, product safety, commercial performance, channel service and digital behaviour.

Dimension	Core measures	Indicative pilot gate
Corporate and compliance	Percentage of active SKUs with approved master record, claim set, label, warnings and owner	100% of pilot SKUs
Commercial	Net revenue, gross contribution after channel costs, average order value and cash conversion	Positive contribution by channel after test period
Retail	Units/store/week, reorder rate, on-time-in-full, fill rate, stockouts, returns and expiry	Thresholds agreed from buyer economics before pilot
Customer	Trial-to-repeat, repeat interval, review rate, complaint rate and support resolution	Demonstrable repeat within product-appropriate period

Digital	Qualified organic traffic, product-view-to-cart, checkout completion, assisted conversion and delivery-cost exits	Improving cohort trend, not vanity traffic
Brand	Aided awareness, trust, message comprehension, heritage credibility and consideration	Tested against pre-launch baseline
Safety	Adverse events, product-quality complaints, report completion, escalation time and recall readiness	100% logged and processed within SOP
Governance	Content currency, approval exceptions, outdated records and response to corrections	No unapproved claims live

Targets for rate of sale, acquisition cost, repeat purchase and contribution should be set only after current product economics and pilot channel terms are known.

15. Governance and Team Structure

A transformation of this kind cannot be delegated solely to a design or web agency. Management must appoint an executive sponsor and a transformation lead with authority across legal, regulatory, product, operations, finance, sales and marketing.

Role	Accountability
Executive sponsor	Strategic decisions, funding, stage gates and dispute resolution
Transformation lead	Integrated plan, dependencies, risks, decisions and delivery
Responsible pharmacist / regulatory lead	Product status, claims, safety, labels, pharmacovigilance and release approval
Legal and privacy counsel	Entity, contracts, IP, consumer, advertising, privacy and ecommerce policies
Product/PIM owner	SKU truth, change control and channel data

Finance and commercial lead	Prices, margins, terms, channel economics and business case
Sales/channel lead	Accounts, buyer research, pilot stores, training and sell-through
Brand and content lead	Strategy, identity, content system and asset governance
Digital product lead	Platform, integrations, analytics, CRM and service design
Research partner	Consumer, buyer, competitor and concept validation
Quality/operations lead	Suppliers, testing, production, inventory, fulfilment, complaints and recall readiness

A weekly delivery meeting should manage dependencies, while a monthly steering committee should approve claims, portfolio and investment decisions. No agency should be allowed to publish product copy or health claims without the designated approval trail.

16. Budget Scenarios

The following figures are planning ranges, not supplier quotations. They exclude VAT and assume a South African delivery model using senior specialist partners. Major facility upgrades, product reformulation, laboratory and stability programmes, SAHPRA submission fees, packaging print quantities, inventory, retailer listing fees, consumer media and national sales-force cost are excluded.

16.1 Recommended core transformation budget

Workstream	Planning range
Mobilisation, corporate truth and heritage substantiation	R180,000–R320,000
Product master, commercial audit and claims triage	R260,000–R520,000
Consumer, buyer, competitor and demand research	R300,000–R520,000
Brand strategy, architecture and naming	R180,000–R320,000

Visual identity and design system	R240,000–R450,000
Packaging system and 3–5 hero artworks	R320,000–R650,000
Digital strategy, UX, PIM/CMS and website/trade build	R700,000–R1,350,000
Content, product photography and technical/retailer assets	R300,000–R620,000
Policy, privacy, ecommerce and service design	R180,000–R380,000
Gauteng pilot setup, training, sampling and optimisation	R300,000–R600,000
Programme management, testing and contingency	R300,000–R570,000
Indicative core total	R2,900,000–R5,600,000

16.2 Investment options

Option	Scope	Budget	Appropriate when
Validation first	Corporate truth, SKU and claims triage, commercial audit, market research, hero scorecard and initial brand direction	R650,000–R1,150,000	Management needs evidence before committing to a full rebuild
Core transformation	Full programme through identity, hero packaging, website/trade foundation and Gauteng pilot	R2,900,000–R5,600,000	NMS intends to relaunch professionally over 12–18 months
Accelerated growth	Core transformation plus larger packaging rollout, expanded content, greater retail activation, CRM, media and regional scale	R5,500,000–R9,000,000+	Compliance, supply, margins and working capital are already strong

The recommended commercial structure is stage-gated. Approve the validation-first budget immediately, then release later phases after Gate A and Gate B. This protects the business from funding identity or technology around products that may be inactive, unprofitable or unsuitable for publication.

17. Key Risks and Mitigations

Risk	Likelihood	Impact	Mitigation
Legal entity or 1934 continuity cannot be proved	Medium	High	Use precise qualified wording; retain heritage assets as archival rather than legal claims
Priority legacy products cannot support desired claims	High	High	Select lower-risk hero products, reformulate or reposition; quarantine unapproved copy
Current licences or manufacturing controls are inadequate	Unknown	Critical	Complete licence and quality audit before public launch or retailer outreach
Product data remains fragmented	High	High	Appoint PIM owner and prohibit PDF-to-site migration without validation
Rebrand hides rather than fixes the business	Medium	High	Make proof, quality and governance visible; tie creative approval to readiness gates
Retail expansion consumes working capital	Medium	High	Pilot few SKUs and stores; model settlement, promotion, returns and fill-rate capacity
Ecommerce launches without fulfilment or service readiness	Medium	High	Stage catalogue/stockist first; activate checkout

			only after operational acceptance tests
Heritage positioning feels dated	Medium	Medium	Combine archival authenticity with contemporary legibility and mobile-first service
Customer testimonials create prohibited claims	High	High	Moderate, retain source, prohibit health-outcome amplification and use approved scripts
Indigenous sourcing creates conservation or benefit-sharing exposure	Unknown	High	Verify species, permits, sustainability, supplier rights and benefit-sharing where applicable
International expansion creates regulatory and customs risk	Medium	High	Country-gate products and claims; validate importer, registration, duties and returns

18. First 30 Days: Mobilisation Plan

Days 1–5

Management should appoint the executive sponsor and transformation lead; freeze public reuse of the old product copy; secure domains, hosting, email and social credentials; create the data room; and issue a document and sample request to legal, finance, product, warehouse, sales and regulatory stakeholders.

Days 6–10

The legal and corporate workstream should obtain CIPC and ownership records and resolve current company names, registrations, addresses and directors. The product team should create the provisional SKU register from current stock, packaging and sales systems rather than from the PDFs alone. The regulatory lead should begin a high-risk-claims triage.

Days 11–20

Finance and sales should reconstruct the current product, margin and account picture.

Management interviews should establish strategic priorities and operational constraints. The research partner should finalise the survey, interview guides, competitor set and brand hypotheses. Historical artefacts should be scanned and catalogued.

Days 21–30

The steering committee should review the initial corporate fact sheet, licence map, activeSKU count, claims red list, data gaps, commercial baseline and research fieldwork plan. It should decide whether the project can proceed as a broad transformation or must first become a compliance and operational recovery programme.

Day-30 decision pack

The required pack should contain the approved corporate fact sheet; entity and succession diagram; current domain/contact list; licence and facility map; provisional SKU master; top twenty commercial products by current revenue and contribution; product-claims risk heatmap; research plan and sample; budget refinement; and an updated risk register.

19. Management Decisions Required

The proposal can begin without deciding the final logo, website platform or ecommerce technology. It cannot begin effectively without the following executive decisions:

1. **Who owns the transformation?** Name one executive sponsor and one accountable transformation lead.
2. **What is the primary three-year ambition?** Choose the hierarchy among manufacturing credibility, Nature's Own consumer growth, B2B distribution, raw-herb supply, practitioner engagement, DTC ecommerce and exports.
3. **What is the present legal and operating entity?** Authorise legal diligence and controlled disclosure.
4. **Which products are genuinely active?** Provide current inventory, labels, formulations, registrations, sales and margin data.
5. **What is the launch risk appetite?** Confirm that no historical claim will be reused without product-specific approval.
6. **Is ecommerce an immediate requirement or a gated option?** Base the decision on operational readiness rather than on the old online terms.
7. **What validation budget is approved?** The recommended initial release is R650,000– R1.15 million, subject to supplier quotations and the exact scope of regulatory work.

20. Recommended Next Step

Approve a **ten-week Validation and Direction Programme** covering MP01–MP10. Its purpose is to replace assumptions with an approved corporate fact base, a controlled active-SKU universe, a regulatory and claims heatmap, current commercial economics, market and customer evidence, and a three-to-five-product launch recommendation. At the end of week ten, management will receive a go, revise or stop decision for the full brand, packaging, website and market relaunch.

The key principle is sequencing: **truth before identity, portfolio before platform, compliance before promotion, pilot before scale.**

Appendix A — Document-by-Document Assessment

Document	What it contributes	Principal limitation
Companyprofilebrochures.pdf	April/September 2016 company profile, history, range, pharmacist support, terms and contradictory claims marketing examples	Conflicting contacts/codes and extensive unsupported or commercial
newsiteproducts.pdf	Broad 18-page product-copy with categories, ingredients, packs and selected duplicates legacy 2017 errors warnings	2026 metadata does not draft establish current content;
NMS_TopTrading-Categories_Advertbrochures.pdf	2016-era corporate profile, visual identity, category structure, raw-herb list and pharmacy/health-shop route	Scanned marketing claims, missing dates and conflicting domain/contact data
NMSBestproducts2017.pdf	Product codes, barcodes, NAPPI, pack variants, VAT and Pharmed pricing	Historical price sheet with duplicate versions and no current validity Old VAT, unverified
NMSCorporateProfile..pdf	May 2017 entity/profile, ownership/rights, medical portfolio, rand prices, terms claims and no current product and more than 150 herbs	
prices, legal NMSCurrentPriceList1May2017. worksheet, licence	Wholesale accounts, vendor numbers, retailer listing identity, incomplete NAPPI and	Conflicting status

pdf correspondence and company uncertain listing status details

NMSonlineshopTerms&Conditions.pdf	Historical DTC ecommerce ambition, payment, delivery, returns and support concepts	Unfinished, contradictory and not suitable as a current legal policy
	Detailed 2017 website copy, product names, ingredients, packs, warnings and stock	Duplicate codes, blank fields, high-risk claims and no controlled current status notes
NMSWEBPRODUCTSSECTIONV2.pdf		

Appendix B — Known Internal Contradictions to Resolve

Topic	Conflict
Legal registration	2007/017961/07 versus 1980/008685/07 versus incomplete 2011 number
Legal/trading name	Natural Medicinal Service, Natural Medicinal Services (Pty) Ltd and Natural Medicinal Services TVL. (Pty) Ltd
Ownership	Mvelo versus Myelo Pharmaceuticals Group; undated “recent” acquisition
Swiss distribution	Rights stated as beginning in1970 or 1979; Kernosan also rendered Kennosan
Domain and email	.com and .co.za domains; mweb.co.za, mweb.com, info@ and orders@ addresses
Contact details	Several phone variants and street postcodes
Product codes	Lexan F930/F932/F937; R956 for Arthritis and Boldo; F920 for Hermes Nos.6 and 8; F908 for Asthma & Bronchitis and Blood Pressure
Product names	Cystosan/Cystocan, Fo-Ti-Tieng/Fo-Ti-Teng, D4/“04” Stomach

Availability	No.10 tea, mouthwash and Cardivallin marked unavailable in historic material
Pricing	Conflicting VAT-inclusive amounts, discount calculations and Lexan case pricing
Returns	Seven-day vendor rule versus unclear 14-day online issue-notification rule
Safety	“Safe” and “herbal therefore harmless” versus toxicity, interaction and pregnancy warnings

Appendix C — Preliminary Website Acceptance Criteria

Area	Minimum acceptance condition
Corporate identity	Legal name, registration, address, ownership language and contact details approved
Product publication	Every live SKU has approved record, owner, date, claim set, directions and warnings
Accessibility	WCAG-informed keyboard, contrast, text, form, image and error-handling implementation
Mobile	Core journeys usable on low-bandwidth mobile and common South African devices
Search	Canonical, indexable category and product structure with structured data and no thin duplicates
Ecommerce	Prices, VAT, stock, delivery, payment, returns, support and terms agree across interface and policy
Privacy	Information Officer, processing map, notices, consent, operators, retention and rights routes implemented
Service	Enquiry, order, complaint, adverse-event and product-quality contacts route to trained

	owners
Trade	Verification, price permissions, MOQ, documents and account support tested
Analytics	Consent-aware events, ecommerce or lead funnels, dashboards and data ownership confirmed
Governance	Publication workflow, approvals, version history, review dates and rollback tested

Appendix D — References

- [1]: [Companyprofilebrochures.pdf](#) — NMS company-profile and brochure compilation, April/September 2016 context.
- [2]: [newsiteproducts.pdf](#) — NMS website product-copy compilation; file metadata dated 2026, content largely legacy.
- [3]: [NMS_TopTrading-Categories_Advertbrochures.pdf](#) — company profile, categories, product brochure and advertisements.
- [4]: [NMSBestproducts2017.pdf](#) — historic product and Pharmed price-list extract, effective 1 May 2017.
- [5]: [NMSCorporateProfile..pdf](#) — corporate profile and catalogue, amended May 2017.
- [6]: [NMSCurrentPriceList1May2017.pdf](#) — price list, listing worksheet, vendor directory and licence correspondence.
- [7]: [NMsonlineshopTerms&Conditions.pdf](#) — historic online-shop terms and conditions.
- [8]: [NMSWEBPRODUCTSSECTIONV2.pdf](#) — 2017 internal website product-copy draft.
- [9]: [SAHPRA — Complementary Medicines](#)
- [10]: [Statistics South Africa — 2024 Mid-year Population Estimates](#)
- [11]: [Statistics South Africa — Census 2022 Population Count Results](#)
- [12]: [Insight Survey — Natural and Organic Supplements Seed Strong Growth](#)
- [13]: [Euromonitor — Dietary Supplements in South Africa](#)
- [14]: [Euromonitor — Herbal/Traditional Products in South Africa](#)
- [15]: [African Journal of Primary Health Care & Family Medicine — Concurrent Use of Herbal and Prescribed Medicine](#)
- [16]: [DFFE — Bioprospecting Value Chains](#)
- [17]: [South African Journal of Science — Medicinal Plant Cultivation for Sustainable Use and Commercialisation](#)

- [18]: [Clicks Group — Clicks Business Review 2025](#)
- [19]: [Clicks Group — UPD Business Review 2025](#)
- [20]: [Dis-Chem — Integrated Report 2026](#)
- [21]: [Shoprite Holdings — Supplier Information](#)
- [22]: [World Wide Worx — South African Online Retail Outlook](#)
- [23]: [Faithful to Nature — Health](#)
- [24]: [Essentially Natural — Herbal Teas](#)
- [25]: [Takealot — Sell on Takealot](#)
- [26]: [Consumer Protection Act 68 of 2008](#)
- [27]: [Electronic Communications and Transactions Act 25 of 2002](#)
- [28]: [Protection of Personal Information Act 4 of 2013](#)
- [29]: [Google Search Central — Product Structured Data](#)
- [30]: [WHO — Traditional Medicine Questions and Answers](#)
- [31]: [WHO — Global Traditional Medicine Strategy 2025–2034](#)
- [32]: [Traditional Health Practitioners Act 22 of 2007](#)
- [33]: [SAHPRA — Guidelines for Advertisement of Medicines and Health Products](#)
- [34]: [Natura Laboratory — About Us](#)
- [35]: [Phyto-Force Herbal Laboratories — About](#)
- [36]: [A.Vogel South Africa](#)
- [37]: [Wellness Warehouse](#)
- [38]: [Living Naturally](#)
- [39]: [Solgar South Africa](#)
- [40]: [Escentia Essential Oils and Aromatherapy Products](#)